

D12 – Office Visit Note

M.K. Osei, MD | Internal Medicine | 03 Aug 2020

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 08/03/2020 | Provider: M.K. Osei, MD

Visit Type: New Patient (Transferred Care) / Urgent

Practice: Norfolk Veterans Medical Associates, Norfolk VA

CHIEF COMPLAINT: Exertional chest pressure x 6 weeks. Dyspnea on exertion.

HISTORY OF PRESENT ILLNESS:

64-year-old retired USN Senior Chief Petty Officer presenting with new exertional symptoms. Patient transferred care from Oak Harbor Family Medicine (Dr. P.A. Kim) following retirement September 2015 and subsequent relocation to Norfolk, VA. Outside records reviewed in full.

Reports exertional substernal chest pressure for the past 6 weeks. Provoked by climbing stairs and brisk walking. Relieved with rest within minutes. Mild associated dyspnea with same triggers. No symptoms at rest. No nocturnal dyspnea, no orthopnea, no syncope. No palpitations beyond his baseline atrial fibrillation. No leg pain, no leg swelling. Concerned enough to seek care; not previously evaluated for these symptoms.

CURRENT MEDICATIONS:

- Omeprazole 20 mg PO daily (dose reduced from 40 in Jun 2019 per GI)
- Metoprolol succinate 25 mg PO daily
- Rivaroxaban 20 mg PO daily
- Tamsulosin 0.4 mg PO daily
- Topiramate 25 mg PO daily
- Naproxen 500 mg PO BID PRN
- Flunisolide nasal spray PRN
- CPAP 10 cmH2O nightly

PHYSICAL EXAMINATION:

Vitals: HR 74 | BP 152/94 | RR 16 | SpO2 96% RA | Wt 91 kg

General: Well-appearing retired military male in NAD at rest.

CV: Rate-controlled, regular. No murmurs, rubs, or gallops. JVP normal. No carotid bruits.

Lungs: Clear bilaterally with good air movement.

Abdomen: Soft, non-tender. Prior surgical scars well-healed.

Ext: No edema. Distal pulses 2+ symmetric. No calf tenderness.

IN-CLINIC STUDIES:

EKG (in clinic): Rate-controlled rhythm, no acute ischemic changes.

ASSESSMENT & PLAN:

1. Exertional chest pressure / suspected angina - new symptoms over 6 weeks in a patient with multiple cardiovascular risk factors (chronic A-fib, age 64, retired military service, OSA). EKG without acute changes. Differential includes stable angina, GERD, MSK. Nuclear stress test ordered. Urgent cardiology referral placed to Dr. F. Mensah, who previously cared for this patient at Puget Sound Cardiology and now practices at Norfolk Cardiology Associates. Start ASA 81 mg PO daily and atorvastatin 40 mg PO daily empirically pending cardiology evaluation.

2. GERD - stable post Nissen x2 (Sep 2009, Feb 2015). Continue omeprazole 20 mg daily (dose reduced in 2019 by GI).
3. BPH - stable on tamsulosin. Most recent PSA Mar 2019: 3.2 ng/mL (stable). Continue annual surveillance.
4. Migraine headaches - stable on topiramate 25 mg daily. Average frequency unchanged at approximately every 6 to 8 weeks. No change.
5. PTSD - service-connected, managed by VA Mental Health, Norfolk. Stable on current outpatient regimen per outside records.

Labs/Orders: Lipid panel, HbA1c, BMP, troponin (baseline), CXR

Follow-up: Urgent cardiology. Stress test scheduled. RTC PCP 2 weeks or sooner if symptoms worsen.

Electronically signed by: M.K. Osei, MD

Date: 08/03/2020 16:05

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